Which treatment this guide covers NCCP 00221a covers adults with locally advanced or metastatic NSCLC with an activating EGFR mutation who have not received an EGFR TKI. The separate licensed squamous-cancer indication after platinum treatment is not included in this NCCP regimen or its evidence card.

NCCP 00221 · VERSION 5

Afatinib monotherapy

The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Diarrhoea, dehydration and kidney problems

☐ Mouth soreness and reduced appetite

☐ Liver injury

☐ Eye inflammation

☐ Rash, dry skin and nail changes

☐ Serious lung inflammation

☐ Rare serious skin reactions and bowel perforation

Discuss the aim of advanced cancer control, expected benefit and uncertainty, alternatives including not proceeding, bowel and skin effects, serious lung/liver problems, fertility and pregnancy, daily treatment, monitoring and your right to reconsider.

Individual benefit, alternatives and concerns

Agreed checks and next review

Confirm the activating EGFR test, previous treatments and prescription. Check blood count, kidneys and liver. A heart-pumping test (ECHO or MUGA) is used if clinically indicated. Review lungs, bowel, eyes, skin, all medicines and pregnancy precautions. NCCP specifies blood count, kidney and liver tests monthly. Heart-function tests are repeated if clinically indicated. Report diarrhoea and skin changes early, especially in the first weeks. Scans and cancer reviews follow your Consultant’s plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending